

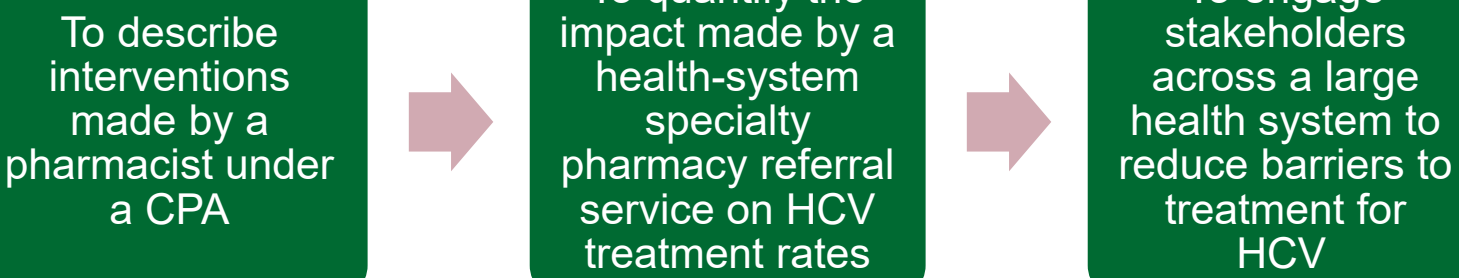
The investigators declare no conflicts of interest



Background

- Approximately 50 million people have chronic hepatitis C (HCV), with 1 million new infections occurring annually. Ohio has an estimated 89,000 people living with chronic HCV, which is likely underreported due to lack of awareness to screen.¹
- Access to direct-acting medications (DAAs) remains low due to a multitude of barriers. Patient navigation programs run by a specialty pharmacy team improves access.²
- An evaluation of the current model of care for patients who test positive for HCV found that system and patient barriers prevented patients from getting treatment. Specialty pharmacists obtained a collaborative practice agreement (CPA), created an HCV Ambulatory service and gathered stakeholders to address each barrier.³

Objectives



Methods

Study Design

•This was an observational quality improvement project of the impact of a Health System Specialty Pharmacy Cascade of Care Referral Service on HCV treatment rates between 10/1/2024 – 3/31/2025

Data

•Data included patient demographics, prevalence of HCV infection, medication data, types of barriers removed, and ultimately treatment rate

Patient Tracking

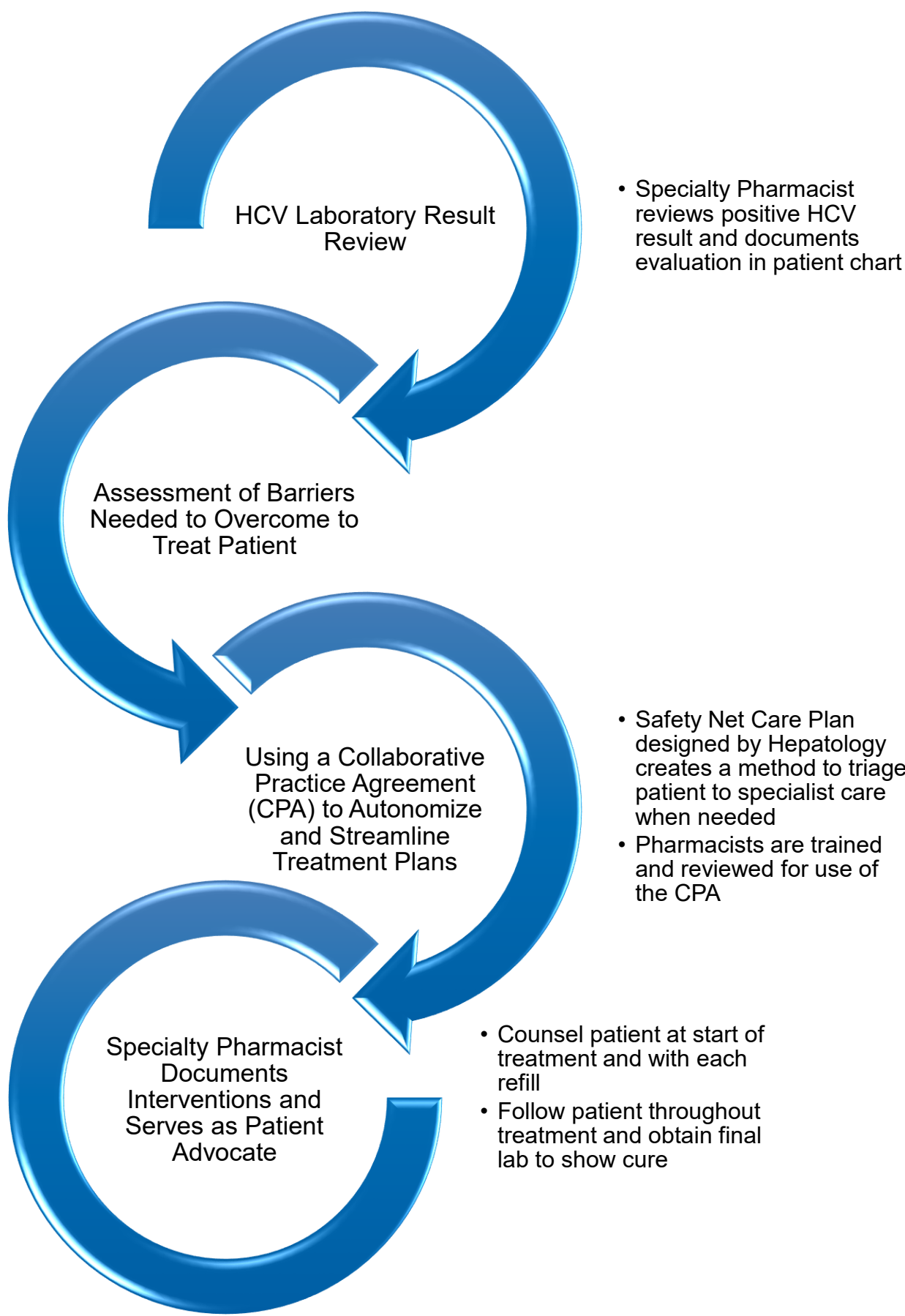
•Enrollment into a patient management system with the electronic chart system served to track patients eligible for HCV treatment and the barriers that were identified and attempts to remove those barriers. Patients were tracked starting with positive test result, throughout HCV treatment and until final documentation of cure

Population

•This analysis included patients who had engaged in care at a Cleveland Clinic facility and had a blood test which was positive for Hepatitis C virus

Definitions

Specialty Pharmacy Workflow



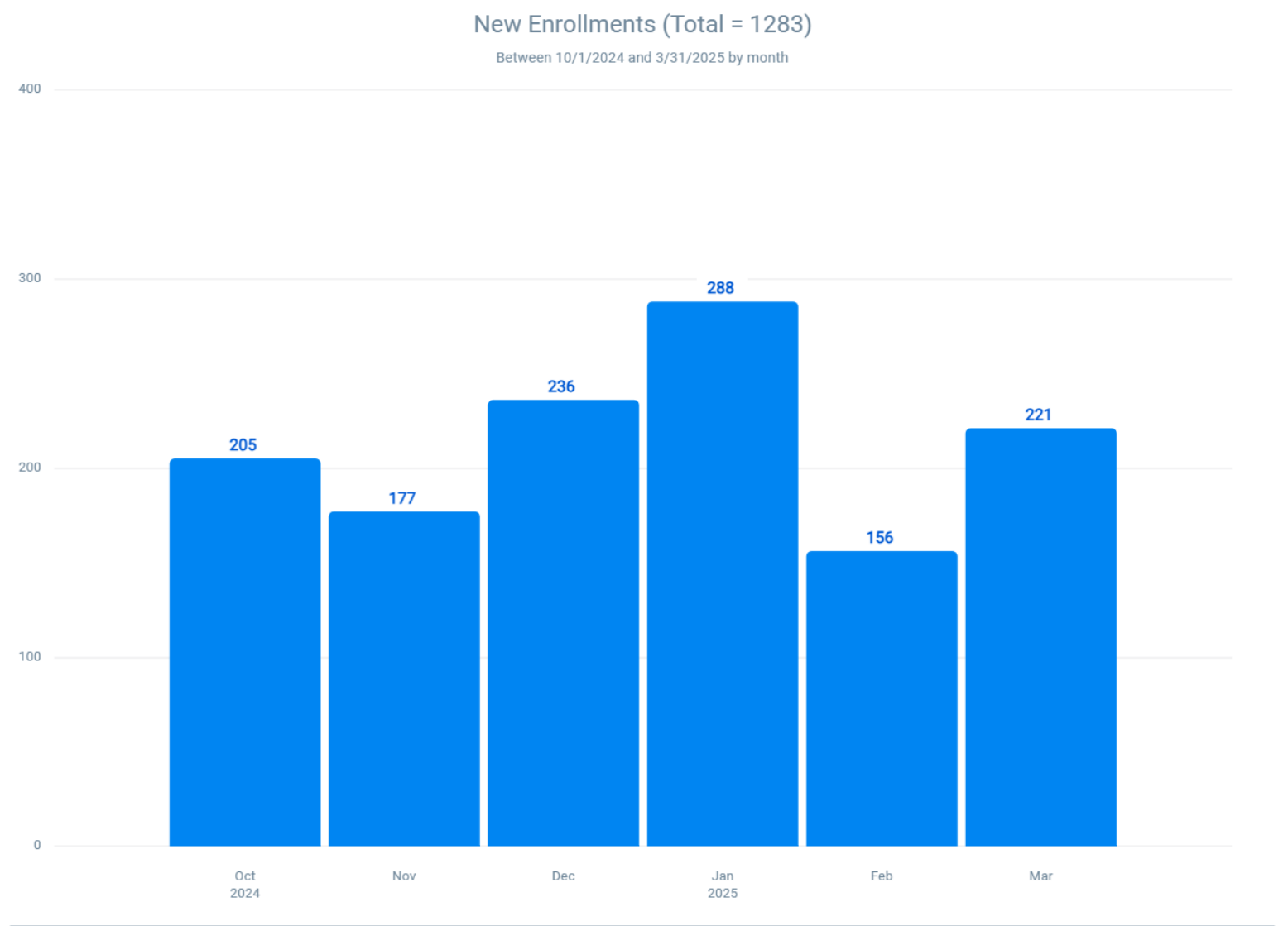
Barriers to Treatment

- Lack of awareness of HCV program
- Not connected to care (no primary care provider)
- Lack of access to a specialist
- Transportation
- Psychosocial
- Substance Use Disorder (SUD) or Alcohol Use Disorder (AUD)
- Cost or lack of insurance
 - Copay assistance availability and eligibility
 - Patient assistance availability and eligibility
- Low health literacy
- Patient lack of follow up with ordered testing or follow up visits

Results

Table 1. HCV Ambulatory Service Referrals

Support Programs	n=1860
Unable to facilitate treatment (ineligible)	613 (33%)
Pharmacist managed treatment via CPA	472 (25%)
Specialist provider prescribed and sent prescription to pharmacy	402 (22%)
Specialist provider managed / pharmacist intervention (barrier to care)	373 (20%)



Description
Newly enrolled patients during the time period in respective support program.
This report accounts for any duplicates from switching support programs (for example if a patient starts in Bucket 2 but then switches to Bucket 4 this report only counts them ONCE).

Total CPA Referrals Received
(10/1/2024 – 3/31/2025)

289

Overall Number of HCV Prescriptions Received

Q4 2023 – Q1 2024

305

Q4 2024 – Q1 2025

399

31% increase

Overall Number of HCV Prescriptions Filled

Q4 2023 – Q1 2024

374

Q4 2024 – Q1 2025

602

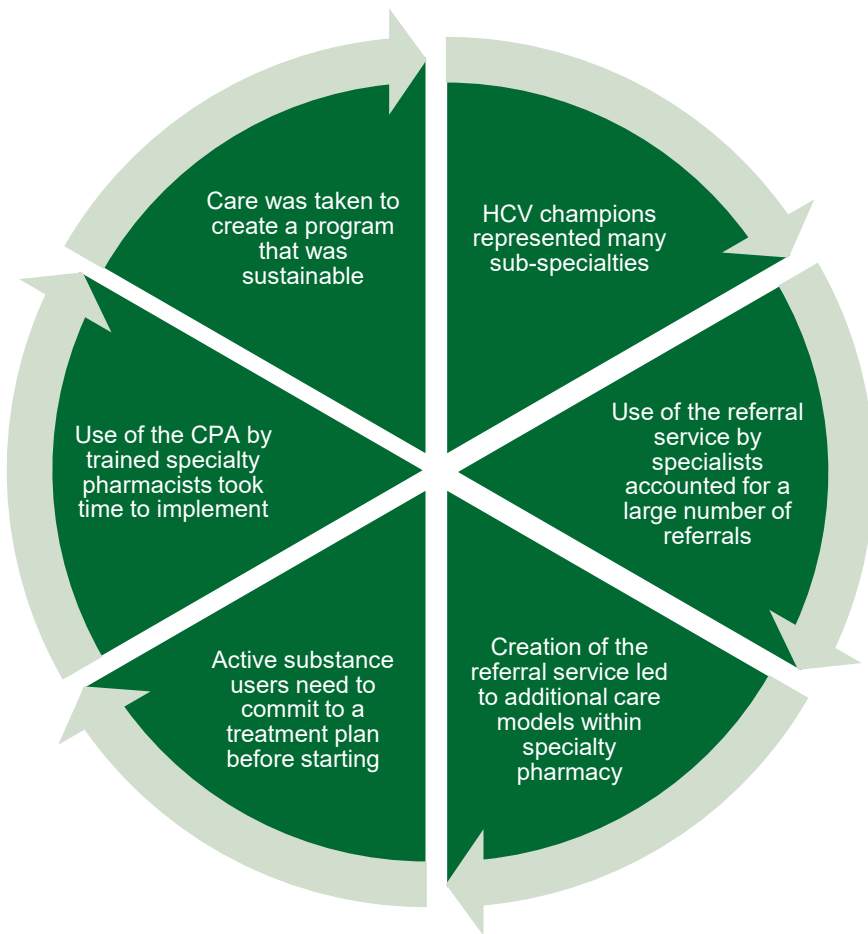
61% increase

Results

End of Treatment Testing

- 97% (125/128) patients successfully completed the cascade of care model

Observations



Conclusions

- Patients reviewed through the cascade of care referral model had high treatment rates
- The health-system specialty pharmacy experienced an increase in volume because of the referral service
- Specialty pharmacy services provided to patients can help to remove barriers to treatment and create easier access to care

References

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